January 19, 2026

Department of Health and Human Services
Assistant Secretary for Technology Policy and the Office of the National Coordinator for Health Information Technology
Attention: Request for Information: HHS Health Sector AI RFI
Mary E. Switzer Building, Mail Stop: 7033A
330 C Street SW, Washington, DC 20201

RE: HHS Health Sector AI RFI

To Whom It May Concern,

Please find attached my response to the RIN 0955-AA13 Request for Information on Accelerating the Adoption and Use of Artificial Intelligence in Clinical Care.

My submission focuses on the importance of auditability for AI-enabled and networked surgical systems and the need for reliable, cross-system reconstruction of procedural events to support safety, postmarket learning, and accountability as these technologies scale.

I appreciate the opportunity to provide input on this important topic and would welcome the opportunity to clarify or expand on any aspect of this response if helpful.

Sincerely,

Binita S. Ashar, MD, MBA, FACS, MAMSE
Founder, Clarity Surgical Advisors LLC

Response to HHS Health Sector AI RFI

Auditability of AI-Enabled and Networked Surgical Systems

As HHS seeks to accelerate the safe and trustworthy adoption of AI in clinical care, a foundational barrier has emerged in advanced, AI-enabled and networked surgical systems. These systems integrate devices, software, cloud platforms, networks, and clinician decision-making in real time. When adverse events, unexpected outcomes, or performance questions arise, it is often difficult to reconstruct what actually occurred.

This is not primarily a data availability problem. Rather, it is an auditability problem. Relevant information is fragmented across manufacturers, hospitals, electronic health records, device logs, and cloud systems, with no reliable way to assemble a coherent, verifiable account of events. This fragmentation constrains patient safety, postmarket learning, payer confidence, and public trust which are all central to HHS's mission.

Centralized control of all data is neither realistic nor desirable. Instead, HHS should elevate auditability as a system requirement for high-risk, AI-enabled care. By auditability, I mean that authorized parties can reconstruct relevant aspects of a procedure across systems using preserved, reliable records under clear rules of access and use.

HHS is well positioned to catalyze progress by coordinating across FDA, CMS, and ONC to define minimum attributes of an auditable procedural record, including traceability, data integrity, continuity across software updates, and authorized access for safety investigations and quality improvement. HHS could further align innovation funding and demonstration programs to prioritize systems that support auditability and encourage interoperability standards that enable cross-system reconstruction of events. CMS, in particular, could incorporate support for auditable records into innovation models or quality programs to better align payment incentives with safer and more learnable AI deployment.

Making auditability a baseline expectation would reduce legal and implementation uncertainty, support safer adoption of AI, and enable continuous learning across health systems without slowing innovation. In short, fragmentation is inevitable; lack of auditability is not. Elevating auditability will help HHS achieve its goals of faster, safer, and more trustworthy AI adoption in clinical care.